

ASSIGNMENT BRIEF

PROGRAMME TITLE	BSc (Hons) Health and Social Care Management - Apr 2022 FY and Apr 2023	MODULE CREDIT VALUE	20
MODULE TITLE	Service Planning and Budgeting	MODULE LEVEL	5
MODULE CODE	LSME510	ACADEMIC YEAR	2024/25
LECTURER	Dr Praveen Sadasivan		
LEARNING OUTCOMES	On successful completion of this module students will be able to: 1. Use appropriate techniques to prepare and lead a planning process to create service direction. 2. Prepare and negotiate service budgets in single and multi-agency contexts. 3. Analyse the context in which budgetary management takes place to ensure efficient reporting and appropriate interventions if there are problems. 4. Identify the link between budgets and service provision using key performance indicators including non-financial measures of performance.		
SUBMISSION DEADLINES	-Assignment 1: Case Study-26/02/2025 4pm -Assignment 2: Case Study-26/03/2025 4pm	ISSUE DATE	09/01/2025
SIGNATURE OF ASSESSOR	Praveen Sadasivan		

MODULE DESCRIPTION

The module combines service planning with budgeting and reflects a growing understanding of the health and social care sector that plans and budgets should be linked if not fully integrated. This approach can improve planning, performance assessment and decision making. The module aims to enable students to understand how to plan services and interpret budget statements as a holistic process. In this way service management can become more efficient and effective, leading to the opportunity to identify waste and cut costs.

ASSESSMENT TASK

There are two assignment for this module:

Assignment 1 - (Case Study - Service Planning and budgeting) 2000 words +/- 10%

Total Weighting: 50%

Intended Learning Outcomes: 1 and 2

Assignment 2 - (Case Study – Budgetary Management) 2000 words +/- 10%

Total Weighting: 50%

Intended Learning Outcomes: 3 and 4

ASSIGNMENT 1 – CASE STUDY- PLANNING AND BUDGETING

This case is about integrating mental health care into the GP surgeries across Cambridgeshire and Peterborough.

Task - Based on the success of this case study, you are required to prepare and lead a planning process to create a similar service for your health and social care organisation. For instance, prepare a plan of how you would consider integrating primary mental health services into care homes for Dementia patients. Prepare an arbitrary budget for this project and explain how you will negotiate this budget with your managers and the GP surgeries.

[Total: 100 Marks]

ASSIGNMENT 1 – INDICATIVE MARKING CRITERIA

	Case Study
A* 80%+ High First	An excellent answer will have the following attributes: - Exceptionally broad factual and conceptual understanding and use of appropriate techniques to prepare and lead a planning process to create service direction. - Exceptionally in-depth knowledge of the preparation and negotiation of service budgets in single and multi-agency contexts - Excellent application of research-informed knowledge to the health and social care context - Exceptionally well structured and well-presented arguments - Correct use of English language - Referencing incorrect Harvard Style
A 70 - 79% First Class	All components will have been submitted, and the remaining criteria will be as noted for the 80%+ marking band. However, in one or two areas, the submission will be open to minor criticism.
B 60 - 69 % Upper Second	A very good answer will have the following attributes - A good factual and conceptual understanding and use of appropriate techniques to prepare and lead a planning process to create service direction. - Good knowledge of the preparation and negotiation of service budgets in single and multi-agency contexts - Identify the challenges and provide recommendations for alternative solutions - Very good application of research-informed knowledge in the health and social care sector. - A well-structured and coherent work written in correct English. - A full list of references with minor errors/omissions. - Full reference accurately in the Harvard style of referencing
C 50 - 59% Lower Second	All components will have been submitted, and the remaining criteria will be as noted for the 60-69% marking band. However, there will be some gaps in knowledge and analysis.
D 40 – 49% Third Class	The answer in this band of marks will have the following attributes: - A basic level of factual and conceptual understanding and use of appropriate techniques to prepare and lead a planning process to create service direction. - Limited evidence of understanding of the preparation and negotiation of service budgets in single and multi-agency contexts - Mainly descriptive work with disorganised or fragmented analysis. - Satisfactory but weak attempt at application of research-informed knowledge in the health and social care sector. - Lack of wider research and weak application of knowledge in the health and social care sector - Only able to make basic recommendations not demonstrating an in-depth understanding - The work will have some noticeable errors in the English Language. - References and citation of all the sources not included Limited reading of secondary sources to be able to substantiate or contradict the arguments given in the source material - Lacks in articulating the key arguments to produce a generalized summary and recommendation. - Satisfactory data visualization and academic report writing - The work will have some noticeable errors in the English Language. - References and citation of all the sources not included
F 30 - 39% Fail	The answer in this band of marks will have the following attributes: - The work shows an insufficient level of factual or conceptual understanding of the subject. - The work is almost entirely descriptive showing little or no evidence of analysis and with unsubstantiated opinions and no application of knowledge - Poor application of research informed literature - Structure very weak or lacking

	- Many mistakes in grammar or sentence construction. - Underdeveloped academic style of writing - The references list not complete and not in the Harvard referencing style.
F 0 – 29% Poor Fail	- Largely inaccurate or irrelevant material. Little or no evidence of factual or conceptual understanding of the subject or of reading or research. - The work shows no evidence of analysis and contains unsubstantiated opinions and no application of theory - Very poor application of research informed literature. - Structure very weak or lacking. - Many mistakes in grammar or sentence construction. - Citations and bibliography are incorrect or missing. - The work may be incomplete or too brief.

ASSIGNMENT 2 – CASE STUDY- BUDGETARY MANAGMENT

This case study about introducing Primary Care Mental Health Service to GP surgeries of Cambridgeshire and Peterborough suggests that their key performance indicators (KPI) are at a good level and also have effective budgetary management behind their success.

Task- As per the case study and your research, identify the possible KPIs, the link between budgets and KPIs, and analyse how efficient reporting and appropriate interventions accordingly have taken place to ensure a smooth running of this new service.

[Total: 100 Marks]

ASSIGNMENT 2 – INDICATIVE MARKING CRITERIA

	Case Study
A* 80%+ High First	An excellent answer will have the following attributes: - Exceptionally broad factual and conceptual understanding and analysis of the context in which budgetary management takes place to ensure efficient reporting and appropriate interventions if there are problems. - Exceptionally in-depth analysis of the link between budgets and service provision using key performance indicators including non-financial measures of performanceExcellent application of research-informed knowledge to the health and social care context - Exceptionally well structured and well-presented arguments with logical Reasoning. - Correct use of English language - Referencing in the correct Harvard Style
A 70 - 79% First Class	All components will have been submitted, and the remaining criteria will be as noted for the 80%+ marking band. However, in one or two areas, the submission will be open to minor criticism.
B 60 - 69 % Upper Second	A very good answer will have the following attributes - A very good factual and conceptual understanding and analysis of the context in which budgetary management takes place to ensure efficient reporting and appropriate interventions if there are problems. - A very good analysis of the link between budgets and service provision using key performance indicators including non-financial measures of performance - Very good use of research informed literature. - Identify the challenges and provide recommendations for alternative solutions - A well-structured and coherent work written in correct English. - A full list of references with minor errors/omissions. - Full reference accurately in the Harvard style of referencing
C 50 - 59% Lower Second	All components will have been submitted, and the remaining criteria will be as noted for the 60-69% marking band. However, there will be some gaps in knowledge and analysis.
D 40 – 49%	The answer in this band of marks will have the following attributes:

Third Class	• A basic level of factual and conceptual understanding and analysis of the context in which budgetary management takes place to ensure efficient reporting and appropriate interventions if there are problems. • Limited evidence of understanding of the link between budgets and service provision • Satisfactory but weak application of research informed literature • Mainly descriptive work with disorganised or fragmented analysis. • Lack of wider research and weak application of knowledge in the health and social care sector • Only able to make basic recommendations not demonstrating an in-depth understanding • The work will have some noticeable errors in the English Language. • References and citation of all the sources not included Limited reading of secondary sources to be able to substantiate or contradict the arguments given in the source material • Lacks in articulating the key arguments to produce a generalized summary and recommendation. • Satisfactory data visualization and academic report writing • The work will have some noticeable errors in the English Language. • References and citation of all the sources not included
F 30 - 39% Fail	The answer in this band of marks will have the following attributes: • The work shows an insufficient level of factual or conceptual understanding of the subject. • The work is almost entirely descriptive showing little or no evidence of analysis and with unsubstantiated opinions and no application of knowledge • Structure very weak or lacking • Many mistakes in grammar or sentence construction. • Underdeveloped academic style of writing • The references list not complete and not in the Harvard referencing style.
F 0 – 29% Poor Fail	• Largely inaccurate or irrelevant material. Little or no evidence of factual or conceptual understanding of the subject or of reading or research. • The work shows no evidence of analysis and contains unsubstantiated opinions and no application of theory • Structure very weak or lacking. • Many mistakes in grammar or sentence construction. • Citations and bibliography are incorrect or missing. • The work may be incomplete or too brief.

Case study

Adapted from <https://www.longtermplan.nhs.uk/case-studies/gp-based-care>

GP-based care breaks down barriers between physical and mental health

Reg: Integrating Mental Health Care / Facility to GP surgeries across Cambridgeshire and Peterborough

Mental health is a priority for the NHS over the next 10 years, with the ambition to deliver high-quality mental health care for everyone, when and where it's needed. Under the NHS Long Term Plan people with moderate to severe mental illness will access better quality care across primary and community teams, have greater choice and control over the care they receive, and be supported to lead fulfilling lives.

Specialist mental health practitioners are providing enhanced support for patients in all GP practices across Cambridgeshire and Peterborough. The Primary Care Mental Health Service ensures patients can talk to mental health professionals in the familiar environment of their own GP practice.

Covering all GP surgeries in Cambridgeshire and Peterborough, the Primary Care Mental Health Service ensures patients can access prompt advice and support for both their mental and physical health needs in a familiar community setting. The integrated nature of the service sees GPs, specialist mental health practitioners, recovery coaches, peer support, social care and third sector staff working in partnership to provide overall care for the patient.

This initiative started in December 2017 and focuses on people aged 17 to 65 with mental ill-health. Promoting early assessment, treatment and onward referral in the community, the core specialist mental health service is provided by Cambridgeshire and Peterborough NHS Foundation Trust (CPFT).

A patient requiring routine, planned care or assessment will initially see their GP who will seek advice from the Primary Care Mental Health Service, or use an electronic referral system to arrange for the service to contact the patient. The service will arrange a face-to-face assessment or give the patient advice by phone. If they are in mental health crisis, they will be supported by the trust's First Response Service. The patient meets Primary Care Mental Health Service staff in his or her local GP surgery, meaning they are in a familiar environment and travel is reduced. Each GP practice in Cambridgeshire and Peterborough has clinical space for practitioners to run clinics on designated days during the week. Senior clinicians are also available to see people with complex presentations.

Prevention and early intervention

Patients are supported to focus on achievable goals and where appropriate access local community resources. The service aims to move away from purely medical to evidence-based treatment, care and support where prevention and early intervention are key.

Ree Wood, manages the Primary Care Mental Health Service. "The service promotes a patient-centred joint approach to physical and mental health, and supports patients to step down from specialist mental health services. Patients' social care needs are also considered as part of this integrated approach," Ree said. "Our new service has rolled out in phases across the county over 18 months. Our mental health teams, GP practice staff, social care staff, Cambridgeshire and Peterborough Clinical Commissioning Group, and third-sector colleagues have worked tirelessly to get the service embedded in primary care."

As part of the service consultant psychiatrists offer 'virtual clinics' to GP practices, offering guidance to primary care staff on mental health presentations. A CPFT consultant psychiatrist said: "I have been visiting the surgeries in my patch every eight to 12 weeks to meet with the GPs and discuss patients they are concerned about, but also to do some short talks on relevant topics such as prescribing in personality disorders and strategies for drug treatment of depression. Referral numbers from these surgeries have decreased and GPs report they are keen for us to continue visiting as they find it educational and supportive."

Patient viewpoint

Steve – not his real name – was offered an appointment with the Primary Care Mental Health Service when he consulted his GP over self-harming behaviour and suicidal thoughts which he linked to factors including childhood trauma. He was seen promptly by his GP which prevented his mental health from deteriorating further into crisis.

"I was offered an appointment with the Primary Care Mental Health Service team at my local surgery," he explained. "The location was good in terms of a short travel time and the familiar setting helped reduce my anxiety. The practitioner listened to my problems and explained how she could help and what the plan would be. She was approachable, easy to get on with and I felt comfortable telling her things I wouldn't tell many other people. The practitioner gave me some sound advice and there was enough time in the consultation to cover everything."

It was agreed Steve would self-refer to Mind to access group talking therapy. He said he understood the plan drawn up jointly with the mental health practitioner and was happy with the outcome. He rated the Primary Care Mental Health Service as "excellent". "I feel in a strong position to move forward and feel more positive about returning to work," he added.

A model for other NHS trusts

CPFT's Director of Operations, Julie Frake-Harris, added: "Our Primary Care Mental Health Service is quite unique in the country and we have received a lot of attention from other NHS Trusts who are looking at our model." She said key to setting up the service had been forging strong, trusting relationships with primary care colleagues, commissioners, third-sector organisations and community groups. Clarity around expectations of the service had been important in building relationships, integration and ensuring sustainability. Julie added: "There has been a need for a wholesale culture change across all sectors which we have addressed in a phased programme. Culture change is ongoing."

Evaluation report

An evaluation report (November 2018) by an independent consulting company into the impact on different groups said the Primary Care Mental Health Service “is a value-adding addition to the mental health support landscape in Cambridgeshire and Peterborough”. York Consulting highlighted a range of positive feedback following interviews with GPs, patients and mental health staff:

- 1) the service is making a positive contribution to patient wellbeing and the management of mental health conditions
- 2) the service is helping to break down barriers between primary and secondary care, and between physical and mental health provision
- 3) patients not having to access secondary care and getting the support they need in primary care
- 4) shorter waiting times for patients accessing the service and high levels of patient satisfaction.

The consultancy said the findings were “testament to the hard work that has gone into the design and day-to-day delivery of the Primary Care Mental Health Service.”

Cambridgeshire GP Emma Tiffin is the Clinical Mental Health Lead for the county’s Sustainability and Transformation Partnership. She said the Primary Care Mental Health Service is designed to ensure a seamless pathway for patients and a sustainable mental health system for the long-term. “Quality care and good patient outcomes are key to what the Cambridgeshire and Peterborough mental health system aims to deliver and this evaluation supports our aims,” she said.

The Results so far against the Key Performance Indicators (KPIs)

- Over 100,000 users have benefited from this integrated services so far. They proposed to achieve about 10,000 users for month
- 76,000 patients enrolled as of June 2019 – proposed to achieve 60,000 user enrolment.
- 94,327 children have reported improvement in their anxiety levels by using the services– proposed to achieve 50,000 cases
- >95% of users of the service are very satisfied. Proposed to achieve a 90% positive review.

Budgetary management and control

The success of this project is reliant on effective management and budgeting. The original budget was £1000000 for the projects. The overall budget for the roll-out of the project is not yet estimated. Managers of Cambridgeshire and Peterborough NHS Foundation have been compelled by their budget management strategies to think about the future, look ahead and set out the detailed plans to achieve the budget targets for each department. There is coordinated communication among key staff who are responsible for budget targets and this provides the basis for the budget performance appraisal. Due consideration is given to external pressures and the efficacy of reporting over-spending and the introduction of waste and fraud reduction interventions. The financial performance is analysed regularly against intended performance indicators. Hence the budget is a yardstick against which actual performance is measured and assessed. Departures from budgets are then investigated and the reasons for the differences are divided into the controllable and uncontrollable factors and this encourages remedial actions to be taken

Further information: Cambridgeshire and Peterborough NHS Foundation Trust: Primary Care Mental Health Service.

<https://www.haypeterborough.co.uk/support/nhs-cpft-primary-care-mental-health-service/>

End of Case Study

COMPLETING THE TASKS

In order to complete the Assignments 1 and 2, you will need to research different information sources such as textbooks, journals, articles and the internet. You are also required to develop skills in the analysis of information. Analysis requires you to critically examine different aspects of a topic and identify relevant issues. Discussions with your peers and friends will also help you to understand the relevant data analysis and presentation techniques that can be applied to different perspectives of the collated data provided.

Please Note: Make use of information on assignment preparation and command verb usage in your Canvas Platform.

HELPFUL INFORMATION

Core Texts

- Glennerster, H. (2023). *Social Service Budgets and Social Policy: British and American Experience*. United Kingdom: Taylor & Francis.
https://www.google.co.uk/books/edition/Social_Service_Budgets_and_Social_Policy/nSHcEAAAQBAJ?hl=en&gbpv=1&kptab=getbook
- Bergeron, B., P. (2018), *Performance Management in Healthcare: From Key Performance Indicators to Balanced Scorecard*. Boca Raton, FL, CRC Press
- Taylor, B. and Devine, T., 2020. *Assessing needs and planning care in social work*. Routledge.Walley,
- Harrison, J.,P. (2020) *Essentials of Strategic Planning in Healthcare*, Third Edition. Gateway to Healthcare Management- Perlego

Journal Articles

- Khashoggi, B.F. and Murad, A., 2020. Issues of healthcare planning and GIS: a review. *ISPRS International Journal of Geo-Information*, 9(6), p.352.
- Menges, D., Ballouz, T., Anagnostopoulos, A., Aschmann, H.E., Domenghino, A., Fehr, J.S. and Puhan, M.A., 2021. Burden of post-COVID-19 syndrome and implications for healthcare service planning: A population-based cohort study. *PloS one*, 16(7), p.e0254523.
- P., Silvester, K., & Mountford, S. (2006b). Health-care process improvement decisions: A systems perspective. *International Journal of Health Care Quality Assurance*, 19 (1), 93–104.
- Appleby, R.; Crawford, R.; Emmerson, C.; (2009) *How cold will it be? Prospects for NHS funding: 2011-2017*. The King's Fund: London, UK.

Websites and other relevant sources of information

There are many relevant materials from the website that support this module. Students are encouraged to read widely. A few examples are as follows:

- The Kings Fund. (2015). Health and social care: Three priorities for the new government. Available at: <http://www.kingsfund.org.uk/projects/new-gov/three-priorities-new-government>.
- The Nuffield Trust- NHS Timeline Available@ <https://nhstimeline.nuffieldtrust.org.uk/1960s.html>
- KingsFund (2019): NHS Long term plans explained. available at <https://www.kingsfund.org.uk/publications/nhs-long-term-plan-explained>
- KingsFund (2019): NHS five year forward view: Our work looking at the vision for the future of the NHS, available@ health and care <https://www.kingsfund.org.uk/topics/nhs-five-year-forward-view>
- McKenna, H. (2017), *Brexit: the implications for health and social care*. London, Kings Fund. At: <https://www.kingsfund.org.uk/publications/articles/brexit-implications-health-social-care>

REFERENCING YOUR WORK

References to relevant academic theory and research findings should be provided and cited appropriately using the [Harvard system of referencing](#). Examples of this referencing style is as follows:

Book:

Pears, R. and Shields, G. (2016) *Cite them right: the essential referencing guide*. 10th edn. Basingstoke: Palgrave Macmillan

Journal article:

Petit, C. and Sieffermann, J. (2007) 'Testing consumer preferences for iced-coffee: Does the drinking environment have any influence?', *Food Quality and Preference*, 18(1), pp.161-172

Author/s name and initials are listed first, followed by the year of publication in brackets. Then there is the title of the article in single inverted commas and the journal where the article appears, which is in *Italics*. Finally, state the volume and issue number (in brackets) along with the pages where the article can be located. Include at least two in-text citations and references in each assessment criterion. Further information on the Harvard System of referencing is on your Canvas Platform.

Referencing your use of Generative AI

You must describe how the information or material was generated (including the prompts you used), what the output was and how the output was changed by you. You should use the following format of wording, depending on the nature of use:

- The following prompts were input into [AI system]: [List prompt(s)]
- The output obtained was: [Paste the output generated by the AI system]
- The output was changed by me in the following ways: [explain the actions taken]

Here is an example of how to include the use of AI within your references:

OpenAI (2022) ChatGPT: Optimizing Language Models for Dialogue. Available at <https://openai.com/blog/chatgpt/>. (Accessed: 30 June 2023)

You also need to reference the use of AI in-text. Here is an example of how you should write this: (OpenAI, 2022)

Note: Do not use Wikipedia as a source of reference

For further information on Harvard referencing visit link to academic malpractice.
<https://canvas.lsmc.ac.uk/courses/114/pages/study-skills>

Note: Do not use Wikipedia, tutor2U or non-academic websites as a source of reference.

ACADEMIC MALPRACTICE

You are required to work independently when preparing this assignment and you are reminded of the need to avoid the risk (intentionally or unintentionally) of committing academic malpractice. In particular, presenting another learner's work as yours or taking information from any sources without acknowledging the source correctly can constitute academic malpractice. You are expected to submit your work using the Turnitin software that is provided by the school.

Please note that your work may be subject to penalties and/or cancellation if academic malpractice has taken place. The Turnitin similarity report can highlight where academic sources have not been referenced appropriately/effectively and this could result in a concern being highlighted around possible plagiarism (one form of academic malpractice where sources have been used without proper acknowledgement).

Although it is not inappropriate to generate ideas for assignments, presenting an assignment generated solely by

Artificial Intelligence (AI) is similar to contract cheating because the work submitted will not be your original work and the academic malpractice policy will apply.

Make sure you read over your work carefully and ensure that all sources of information have been acknowledged to avoid any untoward investigations that would result in a delay in your achievement of the module. Further information on academic malpractice (including plagiarism and the use of AI) and potential consequences are available in your student handbooks.

You must acknowledge using AI by naming the tool and how it was used within your references. You should use one of the following options:

- No content generated by AI technologies has been presented as my own work.
- I acknowledge the use of [insert AI system(s) and link] to generate materials for background research and self-study in the drafting of this assessment.
- I acknowledge the use of [insert AI system(s) and link] to generate materials that were included within my final assessment in modified form.

Please ensure that you include one of the statements above in the final version of your work that you submit. We suggest that this statement is included within your References or Bibliography section towards the end of your work.

Link to Academic Malpractice <https://canvas.lsme.ac.uk/courses/114/pages/study-skills>

OR <https://lsme.ac.uk/wp-content/uploads/college-policies/LSME-PolicyAcademic-Misconduct-Policy-and-Procedure.pdf>

PRESENTATION

Present a document with an equivalent word count of 2000 words (+/- 10%) (case study 1) and 2000 words (+/- 10%) (case study 2) excluding references, bibliography, images, diagrams, table and appendices. The word count should be stated in the assignment cover sheet and please note you will be penalized for exceeding the word limit. Work must be submitted in a folder, word- processed in a suitable format of 12-point font, 1.5-line spacing and pages numbered

SUBMISSION

When submitting your assignment, you must include:

- An Assignment Submission Sheet/ Assignment Front Cover
- Other documents required by your lecturer as evidence of achievement.
- All assignments should be submitted via the VLE.

Submission deadlines must be strictly observed. Therefore, disciplined time management is very important when producing this assignment. Failure to meet deadlines will be considered as a failure by non-submission. You will only have the opportunity for one resubmission of a different assignment. Refer to pages 22-24, Sections 3.4.8 to 3.4.11 of the General Student Handbook for Undergraduate Programmes for more information on Assignment Deadlines and Late Submission of Assignments on the VLE.

EXTENUATING CIRCUMSTANCES

Students who experience unforeseen or exceptional circumstances that may impact their ability to submit their assignments on time are encouraged to apply for extenuating circumstances with appropriate documentary evidence in accordance with the college's Extenuating Circumstances Policy. Refer to page 24, Section 3.4.12 of the General Student Handbook for Undergraduate Programmes for more information on Extenuating Circumstances on the VLE.

END OF ASSIGNMENT BRIEF